ANAESTHESIA, CRITICAL CARE & PAIN MANAGEMENT DIRECTORATE

OBSTETRIC ANAESTHESIA WRITTEN CONTROL DOCUMENT

LABOUR ANALGESIA AND THE MANAGEMENT OF ANAESTHESIA FOR CAESAREAN SECTION IN PARTURIENTS WITH COVID-19 – POINTS TO CONSIDER

Labour Analgesia

Options include:

1. Entonox (attach a microbiological filter to the Entonox on demand valve)

2. Diamorphine

3. Fentanyl PCA

4. **Early epidural analgesia is advised in Covid-19** (if not contraindicated as per usual contraindications). This is to reduce the risk of requiring a GA for operative delivery. However, please **check platelet count prior to the epidural procedure** to exclude thrombocytopaenia (a feature in 33% of patients with Covid-19)

Anaesthesia for Caesarean Section

Donning PPE (including long sleeve gown, FFP3, face visor or mask-visor) is mandatory for the anaesthetic management of all parturients suffering from Covid-19. However, it is time consuming and will impact on decision to delivery time for category 1 caesarean delivery, no matter what the anaesthetic technique used. Women and their families should be told about this delay

Regional Anaesthesia

1. Provide epidural or spinal anaesthesia as required (all usual contraindications to neuraxial block apply as normal)

2. Check platelet count prior to spinal /CSE or de novo epidural anaesthesia

General Anaesthesia

Avoid general anaesthesia unless absolutely necessary for standard indications (high risk aerosol generating procedure)

Points to consider:

1. Intubating anaesthetist & anaesthetic assistant to wear full PPE

2. The anaesthetist performing intubation should consider wearing a second pair of gloves, and remove the outer pair once the ETT is secured (since they are likely to get respiratory secretions on their gloves)

3. Follow Covid-19 Pre-Induction Checklist for Obstetric GA

4. Ensure a tight seal during preoxygenation to avoid aerosolisation

5. Use standard obstetric GA rapid sequence induction drugs. However, consider ketamine (1-2mg/Kg) if the patient is critically ill and therefore at risk of cardiovascular collapse following induction

6. Do not use high flow nasal oxygen for pre-oxygenation or apnoeic oxygenation

7. Use of videolaryngoscopy is preferable for intubation

8. Do **not** ventilate until cuff inflation is confirmed

9. Determine the position of the ETT using chest wall expansion & End Tidal CO2 (ie. do not auscultate chest)

10. In case of difficult intubation :
    - Plan B is to use a 2nd generation supraglottic airway
    - Plan C is to use FONA scalpel-bougie-tube

11. Clamp ETT before disconnection

12. Check all breathing circuit connections to avoid inadvertent disconnection

13. For extubation, please minimise the number of staff in the room due to the high risk of aerosolisation

Reference

1. OAA & RCoA Guidance on the Management Obstetric Anaesthesia & Labour Analgesia in Parturients with Covid-19. N Lucas & F Donald on behalf of OAA, 16 March 2020